

جوستاف روسي انترناسيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : هبه عبد العظيم محمد عبد الجواد

المحافظة : بنى سويف الحى / المدينة : بنى سويف

العنوان : بني سويف
تاريخ تسجيل الإستمارة : 27-09-2026

رقم البطاقة : 28106262100284 رقم التلفون : 01129114914

العمر(تاريخ الميلاد) : 26-06-1981 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطه العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي
الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157]
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقيع: _____
التوقيع: _____
التوقيع: _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- /د ----- /د ----- /د.ا ----- /د.ا

قرار اللجنة

التشخيص النهائي :

خطه العلاج :

المستشفى :

جہۃ العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: هبه عبد العظيم محمد عبد الجواد National ID: 28106262100284	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
وزارة الداخلية



هو
عبد العظيم محمد عبد الجواد
م
عاصم
سكّر بنى سويف - بنى سويف



L02647057

٢٨١ ٠٣ ٢٦ ٢١ ٠٠ ٢ ٨ ٤



٢٠٢٦/٠١ ٢٨١٠٢٦٢١٠٠٢٨٤
ربة منزل

انثى مسلمة
جابر عبد الجواد احمد عبد الجواد
البطاقة سارية حتى ٢٠٣٣/٠١/٠٣



Name : AFAF EL SAYED AHMED NOFAL

Date : 15-02-2026 12:29

Branch : Portable

Medical No. : GRI-27101061700504

Ref.Physician Prof :

Ins.Company :

MULTI-DETECTOR C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS FOLLOWING ORAL AND INTRAVENOUS CONTRAST MEDIA ADMINISTRATION REVEALED:

- Clinical data: breast cancer

- A Well-defined cystic hepatic lesion at segment II measuring 7.5x8.5 c with no solid component and no contrast enhancement ...Benign cystic lesion

- Multiple mixed lytic and sclerotic osseous lesion at the dorsal and lumbar vertebral bodies.... metastatic bone deposits

Otherwise;

- Normal lung parenchyma, showing no focal lesions.

- Normal vascular mediastinal structures.

- The trachea, main stem bronchi as well as their lobar and segmental divisions are normal.

- No evidence of mediastinal or hilar nodal enlargement .

- No pleural collections.

- No detectable lesions in the chest wall.

- The intra and extra-hepatic biliary radicles are normal.

- The pancreas and spleen are normal.

- Normal CT features of both kidneys.

- Right and left supra renal glands are normal.

- No CT evidence of enlarged abdominal or pelvic lymph nodes.

- No ascites.

- Scans through the pelvis showed no masses.

Opinion

- Hepatic cyst at left hepatic lobe
- Multiple lytic and sclerotic deposits



Much Obligated;
Dr.Khaled Moghazy,MD
Prof.of Radiology

Name: هبه عبد العظيم محمد		
MR. : 4978		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is receiving Arimidex and Zoladex, with Zoladex scheduled for administration on 18/01/2026, and will commence Zometa today. The patient has a history of left breast carcinoma that was locally advanced, estrogen receptor and progesterone receptor positive, and HER2 2+ amplified on SISH. Neoadjuvant chemotherapy was administered, followed by a left total mastectomy in August 2024. The patient had 2 positive axillary nodes (YPN3) and 19 positive sentinel nodes (YPT2 10/19). Post-operative adjuvant therapy included PHESGO until December 2024, after which the patient transitioned to Arimidex and Zometa at the day-care pharmacy. Arimidex was started today, with the next visit scheduled in one month for Zoladex. Arimidex and supplemental therapy were initiated today, with a 3-month course of Zoladex planned; the next visit is scheduled in three months.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
	الطبيب المعالج : ندى حسن صلاح كامل	
يعتمد ,		



SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Afaf El-Sayed Ahmed Noufal

Date : 14-02-2026 10:01

Branch : Dokki

Medical No. : 316846

Ref. Physician Prof: Goussav Roussey Egypt

Ins. Company : تعاقد مستشفيات

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

- A 55-year-old patient, with history of right breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

Compared to the prior study dated 3-11-2025;

- Newly seen mild tracer uptake at the level of DV11, LV1, LV2
- Still seen residual tracer uptake at left humeral head and few bilateral ribs
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Still seen residual active osseous lesions at left humeral head and few bilateral ribs.
- Newly seen/reactivated few vertebral lesions.

Best regards,

Dr. Alaa Nagi, MD
Nuclear Medicine



MINISTRY OF HEALTH

مركز النيل للاشعة النووية

Name : Maf El-Sayed Ahmed Noufal

Date : 03-11-2025 10:23

Branch : Dokki

Medical No. : 240083

Ref. Physician Prof : Gostaf Rosch Hospital

Inv. Company : نوافذ مستشفى

^{99m}Tc MDPSKELETAL SCINTIGRAPHY

History

- A 54-year-old patient, with history of breast cancer and referred for follow up.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- Compared to the prior scan dated 23/08/2025; still noted residual active lesions at left humeral head and bilateral ribs.
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Rather stationary residual metastatic osseous lesions as described.

Best regards,

Dr. Alaa Nagi, MD
Nuclear Medicine



Ministry of Health
Kingdom of Saudi Arabia

Ministry of Health
Kingdom of Saudi Arabia



MDT form

A- Patient ID:

1- Name: عذرا محمد توفيق

Sex:

3- Medical number:

Femate

83132712

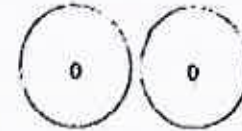
Old Med:

21-02

21/02-2018

2-ID number: 27101061700504

Patient Age: 54 Y



Family history	Family history	Phone	Phone	BMI	BMI	REASON OF MDI	REASON OF MDI
marital status	marital status	NRS score	NRS score	GROUP	GROUP	Date of MDI	Date of MDI
Co-morbidity	Co-morbidity	Menopausal status	Menopausal status	Initial STAGE	Initial STAGE		2025-04-13

Clinical data

Breast Imaging:			Met Workup:		
BX	ER	PR	Her 2 Neu	SISH	KI67
4/8	4/8	4/8	+	+	+
Sonomammography			1) BONE		

Sonomammography					
-----------------	--	--	--	--	--

PATIENT HISTORY					
rt breast cancer 10/2018 T1N0 PORTH + tam 11/2018					
9/2022 VAG BLEEDING txvs ENDOMETRIAL shifted to femara+zoladex 9/2022					
lt humeral mass biopsy: mets adenocarcinoma					
ER G/B PR G/B HER2 0KI67% 15 % GATA +ve					

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN					
for Ibrance,faslodex					
zometa / 3 months					
zoladex					

MEDICAL ONCOLOGY		SIGNATURE	
SURGERY		RADIOLOGY	
Psychiatric		PATHOLOGY	
		Nutrition	

Patient Name : الأستاذة / عفاف السيد احمد نوفل	Request No. : 13009842
Patient No. : 27101061700504	Request Date : 15/03/2025
Path No. : 1030/25	Verification Date : 05/04/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 54 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left humeral soft tissue mass .

Nature of Specimen: Tru-cut biopsy, collected at 13/03/2025.

Gross Examination:

- Four grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) , & GATA3 clone (L50-823)

Microscopic Examination:

- * Histological type: Metastatic adenocarcinoma.
- * IPT (Positive control showed appropriate reactivity):
- GATA3: Positive in tumor cells.
- ER: Moderate nuclear reaction in 50% of tumor cells (score 6/8).
- PR: Moderate nuclear reaction in 50% of tumor cells (score 6/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 15% nuclear reaction of tumor cells.

Diagnosis:

Left humeral soft tissue mass, Tru-cut biopsy:

Metastatic adenocarcinoma of breast origin .

IPT:

- GATA3 : Positive.
- ER: Positive (score 6/8).
- PR: Positive (score 6/8).
- Her2: Negative (score 0).
- Ki67: Positive in 15% of tumor cells.

Verified by :

Dr.Menan Hossam



Patient Name : الأخت / السيدة أسماء محمد
Patient No : 27101061700504
Path No : 1030/25
Sex : Female
Age : 54 Y
Referring Doc :

Request No : 13009842
Request Date : 15/03/2025
Verification Date : 05/04/2025
Financial Category : خدمة المرضى
Department : Out Patient
Registered by : manwa mohamed youss

Pathology Report

* Scoring system "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations. proportion of cells staining (0, no staining; 1, $>1\%$; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%; and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. 2013 IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $< 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $< 10\%$ of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within $< 10\%$ of the invasive tumor cells or complete and circumferential membrane staining that is intense and within $< 10\%$ of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense $< 10\%$ of tumor cells.

Signature
Dr. Rana Fathy
Dr. Menan Hossam

Verified by:
Dr. Menan Hossam

Printing Date : 06/01/2025 11:01:11

Page 1 of 2

Printing By : Dr. Rana Fathy

Patient Name: Asif Sajid Ahmed Naza Date: 21/11/2025
 Age: 34 Sex: M Ref: Asif Sajid
 Med: 25550173 Pat ID: 17225

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS

Known case of right breast cancer.

Findings:

CHEST compared to last study 5/3/2025

- Right breast: operative bed still shows defined irregular shaped soft tissue density lesion resting over the chest wall muscles ? scarring ? recurrence, measures 34 x 22 mm.
- Few left axillary level I and II subcentimetric lymph nodes.
- No appreciable changes regarding subcentimetric left supra-clavicular lymph node measures about 9x6 mm 0.7x0.5cm.
- Still noted right lower lung lobe posterior segment tiny partially calcified pulmonary nodule about 4 mm.
- Right upper and middle lung lobes subpleural reticulations and faint ground glass opacities, likely post radiation sequel.
- Left basal atelectatic bands are noted.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.



II Abdomen & pelvis:

Compared to the previous study dated 20/7/2025, the current study revealed:

- The liver seen mildly enlarged with homogenous CT texture still showing hypodense rather cystic focal lesion occupying segment II and III, no appreciable changes measures about 7.9 x 7.8 x 8.1 cm (compared to 8x7.8x8.5 cm), showing thin septations.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no sizable focal lesions seen. Still noted two splenules.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renal glands.
- Still noted multiple subcentimetric abdominal lymph nodes (para-aortic, mesenteric, right common iliac, left external iliac and bilateral inguinal).
- No ascites.

- Average filling of the urinary bladder with no definite stones or masses.
- No gross adnexal lesions.
- Still noted widespread axial and appendicular skeleton lytic lesions with no associated soft tissue component or structural vertebral collapse.

Opinion:

- Overall Stationary course since the last study

Signature Reham Khagor
 03/11/2025 10:29 AM



Ministry of Health
Government of Saudi Arabia



A-Patient ID:
1- Name: **Dr. Ali Al-Farsi**
2- ID number: **27101651700501**

Sex:

MDT form
3- Medical number:
4- Date:
5- Time: **5:11**

27102212
6- Date:

25/11/2022

Family History

None

Phone

01142955631

DOB

Marital status

Single

MRN Score

CRUI

Comorbidity

None

Menopausal status

Early stage

Reason of Ref

Date of Ref

2022-11-11

Breast Imaging:

Clinical data

Metastasis Workup

DX

ER

A/B

PR

A/B

Her 2 Neu

SI/SI

Ki67

1

N

H

1100:11

Sonomammography

PATIENT HISTORY

1- breast cancer 10/2018 T1N0 P0TH1 11mm 11/2018
9/2022 VAG BLEEDING IVVS ENDOMETRIAL shifted to femara + zoalox 9/2022
11 numeral mass biopsy: mets adenocarcinoma
ER 6/8 PR 6/8 HER2 OKI67% 15 % GATA +ve

MDT OUTCOME & PLAN

for Ibrance, faslodex
zoalox / 3 months
zoalox

MEDICAL ONCOLOGY

SURGERY

Psychiatric

SIGNATURE

RADIOLOGY

PATHOLOGY

Nutrition

10

Patient Name: Afaf Al Sayed Ahmed Nofal	Date : 01/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 55 Year(s)
Medical ID 2589/2018	Pat. ID: 17055

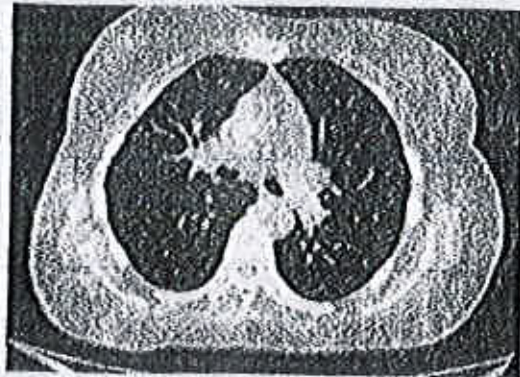
POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS

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- No ascites.



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ربة منزل



أنثى مسلمة متزوجة
جابر عبدالجواد احمد عبدالجواد
البطاقة سارية حتى ٢٠٣٣/٠١/٠٣

٦



Dms

من تلاليل

مركز مؤتمرات + زهور
مونا + ماسون
مونا + ماسون



جمهورية مصر العربية
بطاقة تحقيق الشخصية

هبة

عبدالعظيم محمد عبدالجواد

م عاصم

مركز بنى سويف - بنى سويف

١٩٨١/٠٦

٢٨١.٠٦٢٦٢١.٠٠٢٨٤

L02647057

Patient's Name:

Heba Abdelazim Mohammed

Date:

2026-07-05

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

**RIGHT DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION
SONOGRAPHY REVEALED:**

CLINICAL SUMMARY:

Left MRM

FINDINGS:

RIGHT BREAST:

Scattered fibro-glandular densities, (ACR b).
No spiculations, masses, asymmetry, distortion or suspicious microcalcifications.
UOQ IMLN, complementary US demonstrated intramammary lymph node at 10 oclock zone C measuring 8 x 6 mm with cortical thickness of 3.5 mm
no solid or cystic lesion.
Normal skin/nipple/areola complex.
Non-specific axillary nodes.

LEFT MASTECTOMY BED.

Clear mastectomy bed with no masses or fluid collections.
No pathological regional lymph nodes.

CONCLUSION:

- Right breast (BIRADS 3): UOQ IMLN, as described for follow up
- Clear left mastectomy bed

DR farah elshobaky